



THERIS

ECONOMIC REVIEW BY AMPLIFIER HEALTH

WHAT THIS IS

A line by line economic review of the Theris business. Each clinical service line, the Care Coordinator operating model, each of the four claims on the technology page, and the facility relationship, taken one at a time, asking where reading the voice inside that activity creates an economic case and what it is worth. United States only. Written after your reply, not as an introduction.

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SECTION 01 · HOW TO READ THIS

A Review of Your Business, Not a Pitch

This document does not argue that Theris needs an acoustic model. It works the other way around. It takes each thing Theris already does, one at a time, and asks a single question: is there an economic case for reading the voice inside that activity, and what is it worth?

Six clinical service lines, one operating model, four product claims and one facility relationship. Each gets the same treatment: what it is today, where the economics leak, what an acoustic read adds, and the money line. Where there is no case, or where the evidence is thin, that is stated rather than papered over. Section 06 collects the weak spots in one place.

Everything below is United States mechanics, because Theris operates in seven states with three more launching in Q1 2026 and nowhere else.

THE ONE IDEA UNDERNEATH ALL OF IT

Theris already captures a telehealth session, facilitated by a Care Coordinator who is physically in the room, across every one of these service lines. That audio is currently used once, to deliver and document care. Sona-2 reads it a second time, acoustically, and returns a clinical signal. Every case below is a different answer to the question of what that second read is worth in that specific part of the business.

SECTION 02 · THE SIX CLINICAL SERVICE LINES

Depression, Anxiety, Dementia, Mood, Substance Use, Loss



Depression in High Acuity Patients

TODAY	PHQ-9 monitoring and evidence based intervention, administered by a clinician over telehealth, episodically.
THE GAP	PHQ-9 is self report filtered through a rater on a video call, and it is least reliable exactly here: high acuity, cognitively impaired, and often not candid. Close to half of nursing home residents are depressed. In one Ohio study 48 percent screened positive, 23 percent were receiving no treatment at all, and only 2.5 percent were getting any behavioral therapy.
SONA-2 ADDS	A passive acoustic depression severity read on every session, calibrated against PHQ-9, that does not depend on what the resident chooses to report. It lands in the instrument Theris already charts.

Two revenue paths at once. Enrollment: 99484 at about \$55 per patient per month, or the collaborative care codes at about \$130 per subsequent month. Facility: a PDPM nursing case mix split worth roughly \$30 per resident per day. This is the largest line in the business and the largest case in this document.

Anxiety and PTSD

TODAY	GAD-7 assessment and trauma informed care, covering generalized anxiety, PTSD and social anxiety.
THE GAP	GAD-7 is a point in time questionnaire for a condition that fluctuates day to day. Social anxiety in particular suppresses the very self report the instrument depends on, and late life PTSD is chronically under identified.
SONA-2 ADDS	An acoustic anxiety index on every session, plus PCL-5 calibration, which turns an episodic questionnaire into a continuous measure without adding a single question to the visit.

Anxiety is a qualifying diagnosis for the same BHI and collaborative care enrollment as depression, so it feeds the same \$55 to \$130 per month line from a population that is currently harder to identify. If there is a VA book behind this business, PCL-5 calibration is the single most relevant thing Amplifier has.



Dementia Related Behaviors

TODAY	Person centered, non pharmacological management of BPSD: agitation and aggression, wandering and restlessness, sleep disturbance.
THE GAP	BPSD is managed reactively, after an incident. The intervention is the right one and the timing is the problem, because nobody sees the escalation coming.
SONA-2 ADDS	Clarity, the cognitive model, runs on the same session audio and reads acoustic markers of agitation and cognitive decline ahead of the incident, turning a reactive service into a scheduled one.

This is the most undervalued line. 99483, cognitive assessment and care plan, pays about \$293 non facility or \$170 facility as a single encounter, more than five months of BHI. And the operator side is bigger: see the Five-Star antipsychotic case in Section 05.

Mood Disorders

TODAY	Bipolar disorder management and mood stabilization, with dosing and physiology considerations specific to older adults.
THE GAP	Relapse happens between visits. A switch into hypomania or a depressive turn is visible in days and the next scheduled session may be weeks out.
SONA-2 ADDS	Prosodic change detection against the resident's own prior sessions, which flags the direction of travel rather than the absolute score.

Collaborative care subsequent month billing on a population already enrolled, plus avoided transfer. Honest caveat: acoustic detection of a manic switch is less validated in our own work than depression or anxiety, and this line should be scoped as exploratory in any pilot rather than promised.



Substance Use

TODAY	Age appropriate treatment for alcohol use disorder, prescription medication misuse and addiction recovery.
THE GAP	Identification, almost entirely. Older adult substance use is massively under detected, and the standard screening instruments were not built for this cohort or for a resident whose family is in the room.
SONA-2 ADDS	Acoustic markers associated with sedation and intoxication on a session the resident is already attending for another reason, which is the only realistic way to surface this population at scale.

SBIRT billing through G0396 and G0397, roughly \$29 and \$58 per encounter, is the direct code path, though the identification value is worth more than the code. Honest caveat: this is the least validated of the six for Sona-2 today. Treat it as a research line in a pilot, not a revenue commitment.

Adjustment and Loss

TODAY	Adjustment disorders, loss of independence and end of life support, including grief and bereavement.
THE GAP	These are transition triggered and time boxed. Catching the transition is the entire clinical and commercial game, and the transitions that matter are a new admission, a death on the unit, and a functional decline.
SONA-2 ADDS	Change detection against the resident's own baseline from session one, with no enrollment period, which matters in a population that turns over fast.

This is the enrollment pipeline for everything above it. Among residents admitted without pre-existing depression, 9.3 to 14.2 percent are diagnosed within 90 days and 21.6 percent within a year. Those are new billable episodes that only exist if somebody notices the transition.

SECTION 03 · THE OPERATING MODEL

The Care Coordinator Is the P&L

A dedicated Care Coordinator physically on site, in every facility, is the most expensive fixed cost in this business and the thing that makes it work. Familiar faces improve engagement, the coordinator handles setup, facilitation and documentation, and the operator gets the zero staff burden Theris promises. It is also the number that decides the margin, because everything scales with how many residents one coordinator can carry.



Care Coordinator Throughput

TODAY	One coordinator facilitates sessions for a facility's residents, queuing whoever is scheduled.
THE GAP	Every resident gets roughly the same allocation of coordinator and clinician time regardless of risk. That is the default in the absence of a way to rank them, and it caps residents per coordinator.
SONA-2 ADDS	A risk ranking across every resident in the facility, refreshed every session, so the coordinator queues by who is deteriorating rather than by who is next on the list.

This is the highest leverage number in the whole business. Every additional resident one coordinator can carry drops straight to gross margin, and it needs no new headcount, no new contract and no new facility. It cannot be sized from outside: it needs the current average caseload per coordinator, which is ask number three in Section 06.

SECTION 04 · THE FOUR PRODUCT CLAIMS

Tested Against What the Technology Page Promises

Realtime Analysis and Improved Outcomes

TODAY	Continuous AI analysis during the encounter, longitudinal tracking against baseline, recovery markers, compared against billions of clinical datapoints.
THE GAP	It is one modality. Language and the clinician's own observation both flow from the same evidence, so a second opinion drawn from them is not independent.
SONA-2 ADDS	A genuinely independent second read. Sona-2 analyzes acoustics, not the transcript: how the resident sounds rather than what they said.

Two independent modalities agreeing is a materially stronger outcomes claim than one modality repeated, in an analyst conversation, a payer conversation and a regulatory file. It is also the cheapest credibility Theris can buy.



Maximized Reimbursement

TODAY	Automatic billing code determination, intelligent modifier suggestions, auto generated audit documentation.
THE GAP	It codes a patient somebody already identified. It does not find the patient. Every unenrolled but eligible resident is invisible to a coding engine no matter how good the engine is.
SONA-2 ADDS	A read on every encounter, which surfaces residents who qualify and are not enrolled. The existing Theris engine then does what it already does well.

The formula is (newly enrolled per facility) x (facilities) x (\$55 per month). At 10 residents across 200 facilities that is about \$1.3 million a year on general BHI, or about \$3.1 million on collaborative care at \$130. Only the rates are real numbers. This is the highest value seam in the partnership, because the Theris strength sits directly downstream of the gap.

Increased Utilization

TODAY	10 to 15 minutes saved per session through auto drafted charts syncing to the EHR, sold as see more patients.
THE GAP	That is the supply side of utilization and it is already solved. Freeing an hour does not say whose hour it should become.
SONA-2 ADDS	The demand side. Stratification puts the recovered time in front of the residents where it changes an outcome instead of spreading it evenly.

Same pillar Theris already markets, a lever it does not have yet. Sizes off the same caseload number as Section 03, and the two compound: more time freed, better aimed.

Reduced Liability

TODAY	Multi page audit reports generated for every session, ready for CMS audits.
THE GAP	Every element of that report derives from the clinician's own note. It documents the encounter thoroughly, and it corroborates nothing.
SONA-2 ADDS	One data point in the record that was not produced by the person being audited.

It also supplies the audit trail the coding accuracy argument in Section 05 requires, so the mechanism is already built and this costs nothing to add.



SECTION 05 · THE FACILITY RELATIONSHIP

What the Operator Gets Paid, and Why That Decides Renewals

Theris sells to facility operators and bills insurance. The operator renews the contract. So the strongest commercial argument in this document is not about what Theris earns, it is about what the operator earns because Theris is in the building. Two levers, and the Point Click Care integration means the data path for both already exists.

PDPM Nursing Case Mix

TODAY	Depression identified through the PHQ-9 resident mood interview or the staff assessment is a case mix split in the PDPM Nursing component.
THE GAP	A resident in Clinically Complex with a function score of 0 to 5 classifies at a nursing CMI of 1.62 without depression indicators and 1.87 with them. The gap between those two is a resident who screens positive and is not identified.
SONA-2 ADDS	An acoustic read that routes to a clinician assessment, which either confirms the finding or does not, with an audit trail either way.

The 0.25 CMI difference against an FY2026 urban nursing base of roughly \$122 a day is about \$30 per resident per day, or \$800 to \$900 across a typical covered Part A stay. Theris does not bill it. The operator does, which is exactly why it works as a renewal argument.

Five-Star and the Antipsychotic Measure

TODAY	Theris treats dementia related behaviors with person centered, non pharmacological interventions.
THE GAP	The long stay antipsychotic quality measure moved to a hybrid method in January 2026, combining MDS section N with claims data. The national rate rises from 14.64 percent to 16.98 percent under the new calculation, and it rolls into Five-Star with providers sorted into deciles. Every operator's number just got worse through no change in their own behavior.
SONA-2 ADDS	Earlier detection of escalating behavior, which is what makes a non pharmacological intervention possible instead of theoretical.

Five-Star drives census and Medicare Advantage network inclusion. A service that measurably protects an operator's antipsychotic decile in the year the measure got harder is not a nice to have, it is the reason the contract survives. This is the most timely argument Theris has available right now and it is worth leading with in operator conversations.

THE GUARDRAIL ON BOTH OF THESE, AND IT IS NOT OPTIONAL

Both levers work as accuracy and never as inflation. The value is identifying residents who genuinely screen positive and are currently going unidentified, roughly a quarter of residents in the prevalence data. It is not producing depression codes or gaming a decile. Any version that reads as upcoding assistance fails compliance review at the first sophisticated operator and puts the relationship at risk. The acoustic signal routes to a clinician assessment that stands on its own, and the Theris audit report is where that shows.

SECTION 06 · COST, LIMITS AND THE ASK

What It Costs, Where It Is Weak, What Is Needed

Cost to serve

Haven, the mental and behavioral health model, is \$0.18 per assessment or \$0.12 per minute of audio, scaling linearly with encounter volume. At one assessment per resident per week that is \$9.36 per resident per year, against a PDPM swing of \$800 to \$900 a stay and single encounters worth \$293. A free evaluation tier covers 100 assessments a month. Cost to serve is not the variable that decides any case in this document.

Where this is weakest, stated plainly

- Substance use is the least validated of the six service lines for Sona-2. It belongs in a pilot as a research line, not as a revenue commitment.
- Acoustic detection of a manic switch is less validated than depression or anxiety. Scope the mood disorder line as exploratory.
- The Five-Star antipsychotic argument is causally plausible and not yet demonstrated. It needs an operator willing to measure it, which is a different and slower pilot than the enrollment one.
- Every dollar figure here is a public benchmark. None of it is Theris data, and the spread between the illustrative and the real numbers is the whole reason for the three asks below.

The three numbers

WHAT ONLY THERIS HAS

Facility count	The real number, by state and by facility type
Missed enrollment	Residents per facility eligible for BHI or CoCM, not enrolled
Coordinator caseload	Average residents per Care Coordinator today



The one step that tests all of it

Run Sona-2 against a retrospective slice of telehealth session audio Theris has already captured. No workflow change, no consent change, no Care Coordinator or clinician time. It returns two things: a count of residents the model flags who are not currently enrolled, and agreement statistics against the PHQ-9 and GAD-7 scores already sitting in those charts.

The second output is the one worth optimizing for. Agreement against instruments Theris already trusts is the fastest credible proof available, and it is a validation datapoint Theris keeps regardless of whether anything else here goes forward. Integration when it does: one HTTPS POST after the session, JSON back in seconds, asynchronous and server side, inside the documentation pipeline already running into Athena and Point Click Care. BAA at account creation, audio processed and discarded, nothing stored.

FROM AMPLIFIER HEALTH

The retrospective run costs a data pull and nothing else, and it either produces a missed enrollment number and an agreement statistic that carry the cases above or it does not. Every number in this document is a public benchmark waiting to be replaced by a real one. amplifierhealth.com, docs.amplifierhealth.com, console.amplifierhealth.com.